

I am Suzanne O'Shea. My comments are based on my experience as a long time FDA employee working with these issues, and 9 years of working with a number of tissue manufacturers in private practice. My comments are my own, and do not represent the views of my employer or any client.

I have five points to make today.

1. The Draft Guidance on Minimal Manipulation introduces the concept of “main function for the very first time. The concept does not appear in Part 1271, or in any preamble to any proposed or final regulation.

The Draft Guidance cites page 26749 of the preamble to the May 14, 1998 proposal as support for the assertion that the main function of the HCT/P in the donor determines which definition of minimal manipulation applies. However, the phrase “main function” is never used in the proposal. The closest phrase on page 26749 is “basic function or functions,” which is to be used in the context of determining homologous use.

Creation of an important new concept cannot be done through guidance. I request that if FDA wishes to pursue the main function concept, it do so through notice and comment rulemaking.

2. The Draft Guidance on Minimal Manipulation provides FDA's unilateral conclusions on whether tissues are structural or non-structural.

The guidance process does not provide a sufficient opportunity for industry and academia to provide input into the classification of tissues as structural or non-structural. I recognize that comments may be submitted on Draft Guidances, and I do appreciate this public hearing. However, FDA is under no obligation to articulate a response to comments submitted to a Draft Guidance. FDA is free to maintain its initial position without explaining its reasoning.

I request that FDA's classification of tissues as structural or non-structural be based on articulated reasoning that fully takes into account the views of industry and academia, through notice and comment rulemaking.

3. The Draft Guidance on Minimal Manipulation ignores the reality that some human tissues have both structural and non-structural functionality in the donor.

I recommend that FDA expressly acknowledge the full range of functionality of human tissue in the donor, including the reality that some tissues have both structural and non-structural functionality.

As a specific case in point, FDA stated in a 2001 designation letter that amniotic membrane has non-structural anti-scarring, anti-inflammatory functionality in the donor. FDA now says, without any explanation why it has changed its mind, that amniotic membrane is only structural. I recognize that a designation letter is intended for a specific product, and may not be applicable to all similar products. However, a scientific conclusion about the functionality of a tissue in the donor cannot vary based on the use of the tissue in the recipient.

4. The Draft Guidance on Homologous Use explicitly relies on the classification of a tissue as structural or non-structural to identify acceptable homologous uses.

In creating the homologous use regulations, FDA considered and specifically rejected different definitions of homologous use for structural and non-structural tissues. By importing the concept of “main function”

into the analysis of homologous use, FDA is limiting the range of acceptable homologous uses, contrary to current regulations.

5. FDA has applied the definition of minimal manipulation inconsistently, without explanation.

FDA has concluded that micronized bone is a Section 361 product when intended for use as a bone void filler, even though micronization self-evidently alters the strength and compressibility of bone. It must therefore be the case that FDA has concluded the strength and compressibility of bone are not relevant to bone's utility as a bone void filler.

On the other hand, FDA has concluded that micronized amniotic membrane is more than minimally manipulated when intended for anti-scarring and anti-inflammatory uses, because tensile strength and elasticity are altered. Tensile strength and elasticity are not relevant to the utility of amniotic membrane for anti-scarring and anti-inflammatory uses.

FDA has never explained this discrepancy. I recommend that FDA provide a scientific explanation for the difference in treatment of micronized bone, and micronized amniotic membrane.

Thank you.